

Patient ID: SAllenPeriod: February 6 to March 18, 2024Report Date: March 19, 2024Coverage: 676/993h (68%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm
Very High HR > 110 bpm

Low HR < 45 bpm
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm
High Breathing > 30 brpm

High HR > 100 bpm
Very High Breathing > 40 brpm

Last 24 Hours

Elevated Breathing 8 PM to 9 PM			
Vital Stat	Avg	Min	Max
Heart Rate	59 bpm	45 bpm	84 bpm
Breathing	17 brpm	9 brpm	48 brpm

Limited data in the final 24h (16/24h); values reflect available readings.

Last 24 hours: one elevated breathing episode in the evening, otherwise within normal range.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
8 PM to 9 PM	1h	1	Elevated Breathing	25	12	45	Check SpO2 and lung sounds; assess for fluid overload or early infection

Patient clinical status over 42 days from Feb 06 to Mar 18

■ HR ■ Breathing

Elevated Breathing Feb 18 to Feb 24 (7d)	Elevated Breathing Feb 28 to Feb 29 (2d)	Elevated Breathing Mar 03 to Mar 04 (2d)	High Breathing Feb 22 to Feb 23 (2d)	High Breathing Mar 03 to Mar 03 (1d)	Very High Breathing Mar 03 to Mar 03 (1d)
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23 episodic events Detected, spanning 24h total. Conditions: Elevated Breathing, High Breathing, Very High Breathing. **Priority grade: Low**

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
23	24h	Elevated Breathing	<1	68%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Mar 03	4 PM to 5 PM	1h	1	Very High Breathing	41	28	44	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Feb 22	3 PM to 4 PM	2h	2	High Breathing	31	22	43	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Mar 03	10 PM to 11 PM	1h	1	High Breathing	31	13	45	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Feb 18	7 PM to 8 PM	8h	8	Elevated Breathing	27	9	57	Check SpO2 and lung sounds; assess for fluid overload or early infection
Feb 28	10 PM to 11 PM	6h	5	Elevated Breathing	27	9	54	Check SpO2 and lung sounds; assess for fluid overload or early infection
Mar 03	3 PM to 4 PM	2h	2	Elevated Breathing	26	16	43	Check SpO2 and lung sounds; assess for fluid overload or early infection

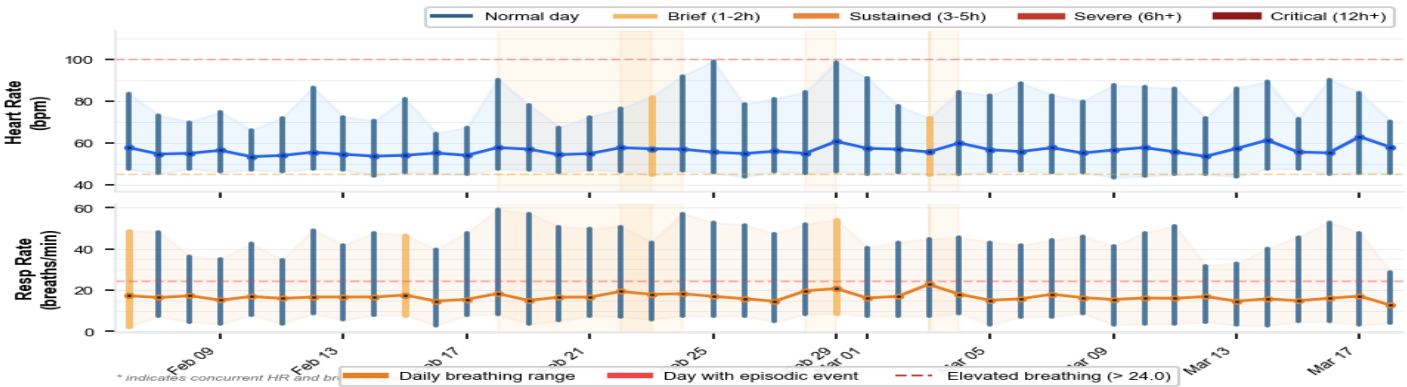
RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

- Elevated Breathing (avg 27 brpm, peak 57 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 31 brpm, peak 43 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Very High Breathing (avg 41 brpm, peak 44 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 6 unstable phases with 19 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

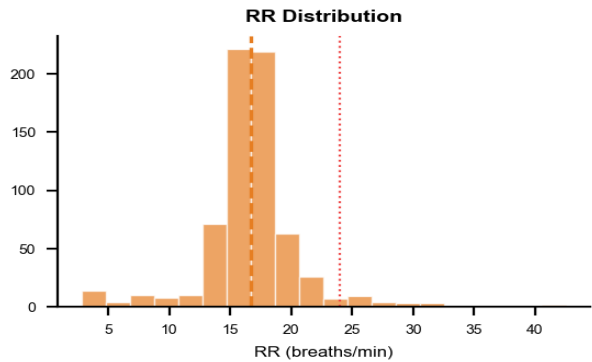
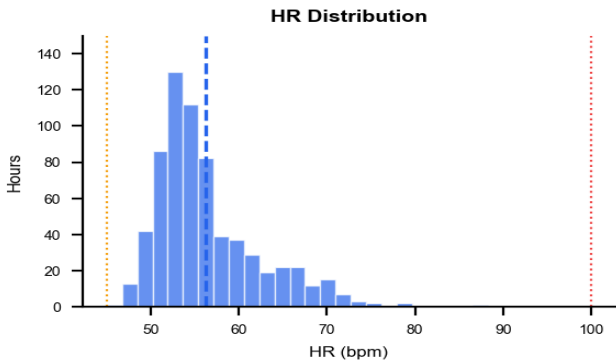
Clinical Pattern Observations:

- 1. **Breathing variability:** RR spread 11 bpm (10 to 21).
- 2. **Clustered pattern:** Episodic events on 5 of 42 days (11%).

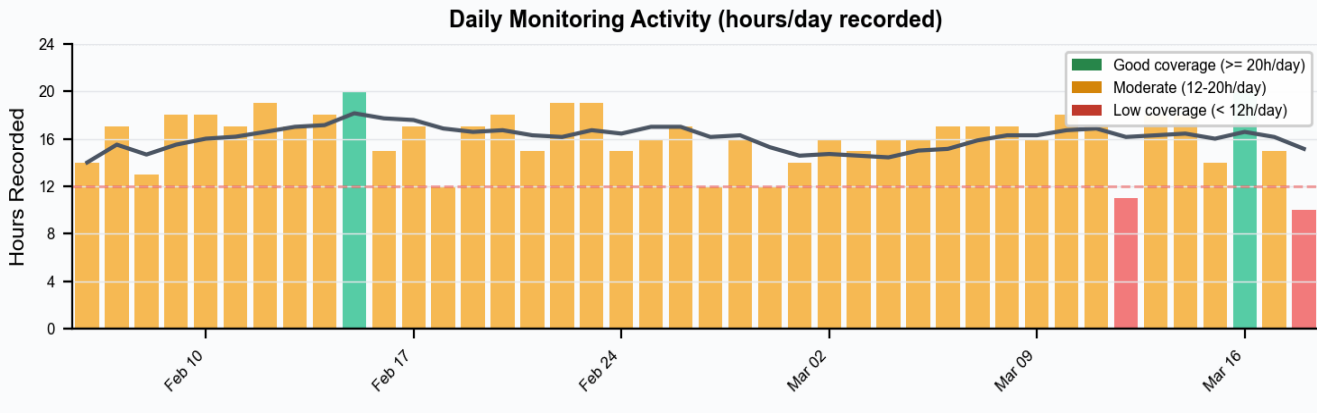


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.